FAX

To: **5122957070**
Company:
Fax: 5122957070
Phone:

From: **mirelesd1**
Fax:
Phone:
E-mail:

NOTES:

Confidentiality Notice: Confidential Health Information Enclosed

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Date and time of transmission:
Number of pages including this cover sheet: 10

Fax

To: MESFIN, FASSIL	**From:** Pam Health at Home - Texas
Fax: 12104504941	**Pages:** 9
Phone:	**Date:** 02-26-2026 11:42 AM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

IMPORTANT - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error please contact the sender and destroy this facsimile.

Order No:485-26056

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
98492469901	02/21/2026	02/21/2026 - 04/21/2026	9604	262009456

6. Patient's Name and Address
ROBINSON, DARRYL N (9604)
3014 OAK HOLLOW DR
New Braunfels, TX 78132
Phone: (830) 388-2757

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 11/09/1961 **9. Sex:** ☑ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Citracal + Vitamin D Maximum 315 mg-6.25 mcg (250 unit) tablet [Amount Of/ Dosage Form: 0 Tablet]	2 TAB	daily	oral	LS
Cyclobenzaprine 5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	3 times a day	oral	LS - PRN/As Needed
Docusate sodium 100 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1 CAP	2 times a day	oral	LS
Dulcolax (bisacodyl) 5 mg tablet,delayed release [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	daily	oral	LS - PRN/As Needed
Duloxetine 60 mg capsule,delayed release [Amount Of/ Dosage Form: 0 Capsule]	1 TAB	2 times a day	oral	LS
Hydrochlorothiazide 12.5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	daily	ORAL	LS
Hydrocodone 10 mg-acetaminophen 325 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	4 times a day	oral	LS - PRN/As Needed
Jardiance 25 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	daily	oral	LS
Lisinopril 20 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	daily	oral	LS
Metformin 500 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	2 TAB	2 times a day	oral	LS
Mounjaro 2.5 mg/0.5 mL subcutaneous pen injector [Amount Of/ Dosage Form: 0 pen injector]	1 PEN	weekly	subcutaneous	LS
Pregabalin 200 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1 TAB	3 times a day	oral	LS
Rosuvastatin 20 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	daily	oral	LS

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
Z48.89	Encounter for other specified surgical aftercare		

12. ICD-9 / 10 CM **Surgical Procedure**

Medicare#:

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Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
M48.04	Spinal stenosis, thoracic region		
G89.29	Other chronic pain		
G95.9	Disease of spinal cord, unspecified		
E11.9	Type 2 diabetes mellitus without complications		
M21.371	Foot drop, right foot		
I10	Essential (primary) hypertension		
E78.5	Hyperlipidemia, unspecified		
F32.A	Depression, unspecified		
K59.00	Constipation, unspecified		
M21.372	Foot drop, left foot		
Z91.81	History of falling		
Z79.84	Long term (current) use of oral hypoglycemic drugs		
Z79.899	Other long term (current) drug therapy		
Z95.2	Presence of prosthetic heart valve		
Z79.891	Long term (current) use of opiate analgesic		
Z98.890	Other specified postprocedural states		
Z51.81	Encounter for therapeutic drug level monitoring		

14. DME and Supplies:

DME: Cane, Dressing Supplies, Grab Bars, Tub/Shower Bench, Walker, Wound Supplies

15. Safety Measures:

911 Emergency / Protocol, Assistance with IADLs, Back care S / P L / S Laminectomy, DIABETIC PRECAUTIONS, Emergency Plan / Measures, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Infection Control Standard Measures and Precautions, Keep Pathways Clear and Safe, Medication Safety / Storage, OPIOID PRECAUTIONS, S/P LAMINECTOMY, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

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16. Nutritional Req.
Sodium Diet: LOW, Carbohydrate Low Diet, No Concentrated Sweets, No Added Salt, HH

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☐ 2- Bowel/Bladder (Incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☑ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☑ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☑ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
SN Frequency: 1W6, 1EOW4,DC OT ORDER,PT Frequency: 1W1, 2W2, 1W1 Starting Week of 02/22/2026

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 300
Weight: < 185 or > 195
Pain: > 7

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SN

BEST PRACTICE FALL PREVENTION:

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

DIABETES:

- SN to instruct patient and family on signs of hyperglycemia to include: Polyphagia- being hungry frequently, pronounced hunger Polydipsia- beingfrequently thirsty, excessive thirst; Polyuria- frequently urinating; Headaches; Difficulty in concentrating; Blurred vision; Weak and tired feeling;Weight loss; Blood glucose level is more than 180 mg/dl.
- SN to instruct patient and family on signs and symptoms of hypoglycemia to include: Confusion, Dizziness, Feeling shaky, Hunger, Headaches,Irritability, Pounding heart; racing pulse, Pale skin Sweating Trembling Weakness Anxiety

GENERAL: BEST PRACTICE DIABETES:

- Clinician to assess/perform monitoring for the presence of skin lesions on the lower extremities and perform patient/caregiver education on diabetic foot care.

SURGICAL WOUND:

- Clinician to assess surgical wound, instruct patient/caregiver on s/s of infection to report to MD.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.

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- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

PT

PHYSICAL THERAPY:

- Establish Home Exercise Program and Activity Program
- Therapeutic Exercises: Strength training
- Gait Training: Even and uneven surfaces and/or natural terrain
- Transfer Training: Sit <-> Stand
- Best Practice: PT to monitor pain and implement strategies to decrease pain as indicated

Assessment Summary

FOCUS OF CARE: SURGICAL INCISION S/P LAMINECTOMY TO T11-T12
CLINICAL SUMMARY: THIS RN WAS GREETED AT THE DOOR BY PATIENT AMBULATING WITH ABNORMAL GAIT UTILIZING WALKER. PATIENT STATES HE HAS HAD MULTIPLE SURGERIES TO NECK SHOULDERS BACK AND HIPS. PATIENT S/P LAMINECTOMY T11-T12 AFTER DECLINE IN MOBILITY, STABILITY, AND NEAR FALLS. ADMISSION AND POC REVIEWED AND AGREED UPON BY PATIENT. PATIENT HAS MEDICAL HISTORY OF DMII THAT HE IS NO LONGER TAKING INSULIN FOR AND IS TAKING METFORMIN AND MOUNJARO AND DOES NOT CHECK BLOOD SUGAR AS PATIENT STATES HIS "DOCTOR TOLD ME I DIDN'T HAVE TO ANYMORE". PATIENT STATES HE HAS BILATERAL FOOT DROP FOR ALMOST 10 YEARS AND IS CURRENTLY WEARING HIGHTOP BOOTS. PHYSICAL ASSESSMENT COMPLETED THIS VISIT. VS WNL WITH THE EXCEPTION OF PAIN 7/10 GENERALIZED. PATIENT STATES HE DEALS WITH CHRONIC PAIN, BREATH SOUNDS CLEAR THROUGHOUT, BOWEL SOUNDS ACTIVE X4, SKIN INTEGRITY INTACT. PATIENT HAS 10CM INCISION TO BACK CONTAINING 21 STAPLES THAT ARE SCHEDULED TO BE REMOVED 2/25. ORDERS ARE TO KEEP DRY AND OPEN TO AIR. MEDICATIONS REVIEWED AND RECONCILED THIS VISIT.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:

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Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 11/09/1961 **9. Sex:** ☑ M ☐ F

1. ROBINSON, DARRYL N is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 02.10.2026 BY FASSIL B MESFIN, MD and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.21.2026.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Multiple emergency department visits (2 or more) in the past 6 months; Decline in mental, emotional, or behavioral status in the past 3 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN,PT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Fall Prevention Program
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110) ; Depression (M1730) ; Home Safety Risks

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

Education needed and provided

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted;
- Patient Chronic and Stable: No
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

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Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG

- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG

- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG

- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG

- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

DIABETES: - LTG

- Hypo/Hyperglycemia sign and symptom free by end of episode.

GENERAL: BEST PRACTICE DIABETES: - STG

- STG-Patient/Caregiver will demonstrate ability to perform proper diabetic foot care as evidenced by Keeping a daily log by 6 weeks.

SURGICAL WOUND: - LTG

- LTG-Patient/Caregiver will be able to identify s/s of infection of surgical wound by EOE. or at end of episode.

SURGICAL WOUND: - STG

- STG-Patient/Caregiver will be able to identify s/s of infection of surgical wound by 4TH VISIT.
- STG-Patient/caregiver will be able to demonstrate independence in wound care by LEAVING OPEN TO AIR . Patient wound will demonstrate evidence of Healing by Primary intention

HOSPITALIZATION RISK FACTORS: - LTG

- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG

- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

HEP: - STG

- STG-Patient and/or caregiver will complete Home Exercise Program with mod I within 2 weeks.

MOBILITY/GAIT: - LTG

- SLTG-Patient will demonstrate improved mobility as evidenced by the increased ability to ambulate with mod I for 500 ft with SPC in order to increase I within 5 weeks. or at end of episode.

PAIN: - STG

- STG-The patient will demonstrate a decrease in pain, limiting participation in ADLs and general mobility as within 2 weeks

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STRENGTH: - LTG

- LTG-Patient will demonstrate improved strength of the B LEs as evidenced by manual muscle test increase from 3/5 to 4/5 in order to increase I within 5 weeks. or at end of episode.

TRANSFER TRAINING: - LTG

- LTG-The patient will improve sit to stand transfer from SBA to mod I using SPC in order to be able to increase I within 5 weeks. or at end of episode.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/21/2026
Signed By: Castro, Paige - RN on 02/21/2026 08:31 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
Fassil Mesfin MD
8300 Floyd Carl Dr 7th floor
San Antonio, TX 78229,NPI: 1992968218, License#: 2013012524,Phone: (210) 567-7463, Fax: (210) 450-4941

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:
03/02/2026

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.